Reproduction in Human Beings

Watch. Predict. Explain.

Original revision guide with 100 practice questions and answer explanations.

Original teaching resource informed by KICD's revised 2024 Grade 8 curriculum. Five introductory missions for strand 2.0, sub-strand 2.3 Reproduction in Human Beings. The curriculum allocates 18 lessons to this sub-strand; this is not a full replacement. Independent prototype, not KICD-endorsed. Teacher review required. The source is linked, never bundled.

KICD Integrated Science Grade 8, revised 2024. strand 2.0, sub-strand 2.3 Reproduction in Human Beings. Source viewer pages 25-26 of 37. Newly written explanations, questions and visuals, not an official assessment or a copy of the source.

This guide contains newly written teaching material. The KICD curriculum PDF is not reproduced or distributed. The original source is available through these external links:

Open the official KICD curriculum listing

View the original curriculum on Google Drive (external)

General education, not diagnosis, contraception prediction or treatment. No personal health information is requested. Seek qualified care for concerns, and prompt help for severe symptoms, exposure, coercion or harm. A teacher should review age suitability.

Use the HTML animation first, try its prediction, then read the optional explanation. All 100 practice questions are below. Short answers are self-assessed; equivalent scientifically correct wording is acceptable. No personal health information should be entered.

Your learning route

1. The menstrual cycle: Describe the main cycle events without treating a model as a personal prediction.

2. Care, support & privacy: Recognise when support is appropriate without diagnosing symptoms.

3. Fertilisation: Describe fusion of sperm and egg at an introductory, non-graphic level.

4. Implantation: Distinguish attachment in the uterus from fertilisation and menstruation.

5. STI knowledge without stigma: Recognise uncertainty, prevention principles and appropriate support.

1. The menstrual cycle

Describe the main cycle events without treating a model as a personal prediction.

The menstrual cycle includes changes in the ovaries and uterus. Menstruation is shedding of the uterine lining with bleeding. Day 1 is the first day of a period.

The lining later builds up, and ovulation is release of an egg from an ovary. If pregnancy does not occur, the lining can shed again and a new cycle begins. Detailed hormonal control is outside this introductory lesson.

A 28-day diagram is a teaching example, not a rule for every person. Cycle lengths, bleeding patterns and ovulation timing vary, especially during adolescence. The model must not be used to predict safe days or diagnose a condition.

Periods are a normal biological process, not a reason for shame, exclusion or teasing. Learn the vocabulary respectfully. This app does not collect dates, symptoms or any learner's personal menstrual history.

Visual mission

A cycle, not a calendar promise

The 28-day wheel is an example; real timing varies.

No personal dates are collected. This cannot predict safe days, diagnose or establish ovulation timing.

Think it through

Explain why a 28-day diagram cannot reliably predict an individual person's ovulation date.

2. Care, support & privacy

Recognise when support is appropriate without diagnosing symptoms.

Cycle variation can occur, particularly in adolescence, but troublesome symptoms deserve support. Pain that disrupts daily activities, concerning heavy bleeding, bleeding between periods or worrying changes should be discussed with a qualified health professional.

A practical school support plan can include access to appropriate menstrual products, clean water, handwashing, private facilities and a trusted adult. Follow product instructions and local health guidance; this lesson does not prescribe medicine.

Do not dismiss severe pain or make a diagnosis from a class discussion. Feeling faint or seriously unwell, or severe bleeding or pain, needs prompt medical attention through a trusted adult or local health service.

Health information is private. A peer can offer respectful support without demanding details, posting photographs, sharing someone else's cycle information or giving unqualified treatment advice.

Visual mission

A support plan, not a diagnosis

Privacy, appropriate facilities and qualified help matter.

Do not enter symptoms or prescribe medicine. Severe symptoms or serious illness need prompt medical attention.

Think it through

Write a general school support plan that includes dignity and access to help without recording anyone's private symptoms.

3. Fertilisation

Describe fusion of sperm and egg at an introductory, non-graphic level.

Sperm and eggs are reproductive cells, also called gametes. Fertilisation occurs when a sperm and egg fuse to form a fertilised egg, called a zygote.

In human reproduction, fertilisation usually occurs in an oviduct, also called a fallopian tube. It is not the same process as implantation in the uterus.

The fertilised egg can develop while travelling towards the uterus. This introductory lesson does not describe the detailed stages of early cell division or hormonal control.

The animation is a non-graphic schematic using labelled markers. Its movement and timing are exaggerated, and it is not a guide to predicting pregnancy or assessing an individual.

Visual mission

Two cells become one fertilised egg

Fertilisation is fusion, usually in an oviduct.

Non-graphic, not to scale and not a personal pregnancy prediction.

Think it through

Explain the difference between ovulation and fertilisation using the correct cell and organ names.

4. Implantation

Distinguish attachment in the uterus from fertilisation and menstruation.

Implantation is attachment and embedding of an early developing embryo, at the blastocyst stage, in the uterine lining. The uterus provides the site; the usual fertilisation site is earlier in the oviduct.

The introductory sequence is ovulation, possible fertilisation, development and travel, then possible implantation. Not every released egg is fertilised, and not every fertilised egg successfully implants.

The lining has a role in supporting early pregnancy. A menstrual-cycle model showing a lining is not itself evidence that implantation or pregnancy has occurred.

The animated marker attaching to the model lining is a simplified illustration, not a medical scan. The lesson omits detailed blastocyst formation and cannot diagnose pregnancy or explain an individual's symptoms.

Visual mission

A different event, a different place

Implantation is attachment in the uterine lining.

Schematic only. Detailed blastocyst formation is omitted; not every fertilised egg implants.

Think it through

Put the model's events in order and explain why a diagram cannot confirm that any event occurred in a real person.

5. STI knowledge without stigma

Recognise uncertainty, prevention principles and appropriate support.

Sexually transmitted infections include HIV, gonorrhoea, syphilis and genital herpes. Many infections can have no noticeable symptoms, and symptoms such as unusual discharge, pain when urinating or sores can have more than one cause. Symptoms alone cannot identify or rule out an STI.

HIV and AIDS are not interchangeable terms: HIV infection can lead to AIDS without appropriate management. Effective professional care matters. HIV is not spread by ordinary casual contact such as hugging, so exclusion and stigma are not justified.

Avoiding sexual contact prevents sexual exposure. Correct and consistent condom use reduces the risk of many STIs but does not eliminate every risk, particularly where uncovered skin is involved. Appropriate health services can advise on testing, treatment and available vaccination.

Do not share needles or handle blood and sharps; seek help from trained adults for an exposure concern. Some infections can also be transmitted during pregnancy or birth or through unsafe blood exposure. Do not diagnose peers, share medicines or assume appearance shows infection status.

A personal concern, coercion or harm should be taken to a trusted adult and suitable health or safeguarding service. This is general education, not individual medical advice. The app asks no questions about personal sexual behaviour or symptoms.

Visual mission

Knowledge without stigma

Appearance is not a test. Privacy and qualified care matter.

General education only. No personal sexual behaviour or symptom information is requested.

Think it through

Explain why 'looks healthy' cannot establish STI status, and name a respectful route to qualified help.

Revision cards

Menstruation

Shedding of the uterine lining with bleeding.

Day 1

The first day of a period, not the first day of every calendar month.

Ovulation

Release of an egg from an ovary; timing varies.

Cycle model

An example, not a personal prediction or safe-days calculator.

Gametes

Reproductive cells: sperm and egg.

Fertilisation

Fusion of sperm and egg, usually in an oviduct.

Zygote

A fertilised egg.

Implantation

Attachment and embedding in the uterine lining.

STI uncertainty

Symptoms may be absent or non-specific; qualified assessment matters.

Respect

Protect privacy, avoid stigma and support access to care.

100-question practice bank

40 multiple-choice and 60 short-answer questions. This is a practice bank, not a standardised exam.

The menstrual cycle

1. What is menstruation?

A. Release of an egg only

B. Fertilisation itself

C. A type of microscope

D. Shedding of the uterine lining with bleeding

2. What is day 1 of a menstrual cycle?

A. Always ovulation day

B. The day an app is opened

C. The first day of a period

D. Always the first day of a calendar month

3. What is ovulation?

A. A pregnancy test

B. Release of an egg from an ovary

C. Implantation in the uterus

D. Shedding a cell wall

4. Is a 28-day cycle diagram universal?

A. No

B. Always

C. Only on paper

D. Only for students

5. Which organ's lining is shed during menstruation?

A. Lung

B. Stomach

C. Kidney

D. Uterus

6. Should this model be used to identify guaranteed safe days?

A. Only with a coloured diagram

B. Only by counting to fourteen

C. No

D. Yes, always

7. Which is a respectful response to periods?

A. Demand personal dates

B. Treat them as normal biology without teasing

C. Exclude people

D. Spread rumours

8. Does this lesson need personal cycle information?

A. No

B. Every learner must submit it

C. Only names are required

D. Only private symptoms are required

9. Name two organs involved in the modelled cycle.

10. Describe what happens to the uterine lining during a period.

11. Describe what happens to the lining after menstruation in the model.

12. What does an ovary release at ovulation?

13. Why is day 14 not a universal ovulation date?

14. What may happen if pregnancy does not occur?

15. Why should the animation be labelled an example?

16. What information should learners not enter into this app?

17. Why is teasing about periods harmful?

18. What is the difference between menstruation and ovulation?

19. Is the menstrual cycle identical to a calendar month?

20. What should guide a personal health concern rather than this diagram?

Care, support & privacy

21. What should happen when pain disrupts daily activities?

A. Always ignore it

B. Diagnose it from a quiz

C. Use a peer's medicine

D. Seek appropriate health advice

22. Which belongs in a respectful school support plan?

A. Teasing

B. Forced personal disclosure

C. Private facilities and access to suitable products

D. Public symptom lists

23. Who should assess concerning heavy bleeding?

A. A guessed social post

B. A qualified health professional

C. A class poll

D. An animation alone

24. Should a learner share someone else's menstrual dates publicly?

A. No

B. Always

C. Only for a joke

D. Only with a quiz score

25. What should guide product use?

A. An invented universal schedule

B. A social challenge

C. A guessed experiment

D. The product instructions and suitable health guidance

26. Which is a safe peer response?

A. Demand private details

B. Dismiss every concern

C. Offer respectful support and help contact a trusted adult

D. Prescribe medicine

27. Does this lesson prescribe pain medicine?

A. Only if symptoms are private

B. No

C. Yes, for everyone

D. Only from a classmate

28. What if someone is faint or seriously unwell with severe symptoms?

A. Get prompt medical help through an adult or health service

B. Wait for a quiz result

C. Post it publicly

D. Treat it as a joke

29. Name one concern that merits health advice.

30. Name two practical school supports.

31. Why should severe pain not simply be dismissed?

32. How can a peer help without diagnosing?

33. Why is someone else's medicine not a classroom recommendation?

34. Why should health information not be posted in a group chat?

35. What should a plan say about handwashing?

36. What is the role of a trusted adult?

37. Why can a class quiz not diagnose menstrual problems?

38. What should someone do about worrying changes?

39. How can a poster be helpful without collecting symptoms?

40. What should happen if a person is seriously unwell?

Fertilisation

41. What are sperm and eggs?

A. Chemical elements

B. Fire fuels

C. Pressure units

D. Reproductive cells

42. What happens at fertilisation?

A. A microscope is focused

B. A cell wall becomes iron

C. A sperm and egg fuse

D. The uterine lining is shed

43. What is a fertilised egg called?

A. Vacuole

B. Zygote

C. Chloroplast

D. Oxygen molecule

44. Where does human fertilisation usually occur?

A. An oviduct or fallopian tube

B. The stomach

C. The lung

D. The outer skin

45. Is fertilisation the same as implantation?

A. Always

B. Only in a diagram

C. Only on day 1

D. No

46. Which event releases an egg before possible fertilisation?

A. Attachment to the uterine lining

B. A diagnosis

C. Ovulation

D. Fusion of sperm and egg

47. What does the animation's marker timing represent?

A. A medical test

B. A simplified teaching sequence

C. A personal pregnancy prediction

D. A universal exact timetable

48. Which claim is appropriate for this introductory lesson?

A. Detailed early division stages are outside its scope

B. Every stage must be guessed

C. A diagram proves pregnancy

D. All pregnancies follow the animation clock

49. Define gamete.

50. Name the two cells involved in fertilisation.

51. Define fertilisation simply.

52. Give another name for an oviduct.

53. What does zygote mean?

54. Towards which organ can the early developing structure travel?

55. Why should the model not predict individual pregnancy timing?

56. Distinguish release of an egg from fusion of gametes.

57. Why is a labelled schematic useful?

58. Does fertilisation usually occur in the stomach?

59. What detail is deliberately omitted here?

60. How should learners discuss reproductive cells?

Implantation

61. Where does implantation normally occur in this lesson?

A. The microscope eyepiece

B. The lung air space

C. The stomach

D. The uterine lining

62. Which structure implants at this introductory level?

A. A chloroplast

B. A metal atom

C. An early embryo at the blastocyst stage

D. An unfertilised egg in every cycle

63. Which usually comes before implantation?

A. Every menstrual bleed

B. Fertilisation

C. A classroom quiz score

D. A completed microscope drawing

64. Does every released egg become a pregnancy?

A. No

B. Always

C. Only in a 28-day diagram

D. Only if a marker moves

65. Can the animation diagnose pregnancy?

A. Always

B. Only on a phone

C. Only after a correct answer

D. No

66. Which is the best distinction?

A. Both mean ovulation

B. Neither involves cells

C. Fertilisation is fusion; implantation is attachment in the uterine lining

D. Both mean menstruation

67. What does the model omit?

A. The fact of biological variation

B. Detailed early developmental stages

C. All labels

D. The idea of a uterus

68. What should guide a personal pregnancy concern?

A. Appropriate qualified health advice

B. The marker's colour

C. A class vote

D. A guessed cycle day

69. Define implantation simply.

70. Name the organ whose lining is involved.

71. Name the early stage mentioned in the curriculum for implantation.

72. State one event that can precede implantation.

73. Why say possible implantation rather than guaranteed implantation?

74. How does menstruation differ from implantation?

75. Why is a coloured lining not proof of pregnancy?

76. What is the usual difference in location between fertilisation and implantation?

77. Why are early division details not shown?

78. What should learners avoid sharing in a public answer?

79. How can a model be useful without being diagnostic?

80. What should be done if the topic raises a personal concern?

STI knowledge without stigma

81. Can an STI be present without noticeable symptoms?

A. Never

B. Only if a diagram is missing

C. Only after a quiz

D. Yes

82. Can symptoms alone confirm which STI someone has?

A. Only in a classroom

B. Only from a photograph

C. No

D. Always

83. Which is an STI named in this lesson?

A. Magnification

B. Gonorrhoea

C. Rusting

D. Osmosis

84. Are HIV and AIDS identical terms?

A. No

B. Always

C. Only in diagrams

D. Only when symptoms are absent

85. Does ordinary hugging transmit HIV?

A. Always

B. Only at school

C. Only while learning

D. No

86. Which statement about condoms is accurate?

A. They diagnose infection

B. They replace all health services

C. They reduce many STI risks when used correctly and consistently, but do not eliminate every risk

D. They guarantee zero risk in every situation

87. Who should assess a personal STI concern?

A. An animation alone

B. A qualified health service

C. A class poll

D. A guessed appearance

88. Which is a respectful response to someone seeking help?

A. Support privacy and access to care

B. Spread a rumour

C. Demand their history

D. Exclude them from class

89. Name two STIs included in this topic.

90. Why does no visible symptom not prove no infection?

91. Give one possible symptom that needs qualified assessment.

92. Explain why appearance cannot establish STI status.

93. Give one prevention principle concerning sexual exposure.

94. Why should needles not be shared?

95. What is a safe response to an exposure concern?

96. Why should peers not share treatments?

97. Give one way to reduce stigma.

98. What is wrong with excluding a learner because of an HIV rumour?

99. What personal information does this app require for the lesson?

100. Who can support someone facing coercion or harm?

Answer key & explanations

1. D. Shedding of the uterine lining with bleeding

Different cycle events have different names.

2. C. The first day of a period

Cycle counting is not calendar-month counting.

3. B. Release of an egg from an ovary

It is distinct from menstruation and fertilisation.

4. A. No

Real cycles and timings vary.

5. D. Uterus

Use accurate, respectful anatomical language.

6. C. No

A simplified calendar is not reliable contraception advice.

7. B. Treat them as normal biology without teasing

Privacy and dignity matter.

8. A. No

It is a general teaching model, not a tracker.

9. Ovaries and uterus.

Their roles are different.

10. It is shed with bleeding.

This is not the same event as ovulation.

11. It builds up again.

The simplified diagram leaves out detailed hormonal control.

12. An egg.

Timing is not identical for every person.

13. Cycle lengths and ovulation timing vary.

A model date is not a personal prediction.

14. The lining can shed and another cycle begins.

Real patterns can vary.

15. It simplifies a variable biological process.

A fixed diagram should not be mistaken for a rule.

16. Personal menstrual dates, symptoms or private health details.

The app is not a clinical or tracking service.

17. It undermines dignity and can discourage seeking support.

Respect helps learning and wellbeing.

18. Menstruation sheds lining; ovulation releases an egg.

The events are not interchangeable.

19. No.

Its timing is biological and variable.

20. Appropriate advice from a qualified health professional.

A learning model cannot diagnose.

21. D. Seek appropriate health advice

Troublesome pain deserves qualified assessment.

22. C. Private facilities and access to suitable products

Practical support can protect dignity.

23. B. A qualified health professional

Symptoms can have different causes.

24. A. No

Personal health information is private.

25. D. The product instructions and suitable health guidance

Products differ and need correct care.

26. C. Offer respectful support and help contact a trusted adult

Support is not diagnosis.

27. B. No

Personal treatment decisions need appropriate guidance.

28. A. Get prompt medical help through an adult or health service

Serious symptoms should not be ignored.

29. For example, pain disrupting activities or concerning heavy bleeding.

A concern needs assessment rather than an online diagnosis.

30. For example, private facilities and access to suitable products.

Clean water, handwashing and a trusted adult also help.

31. It can affect wellbeing and may need medical assessment.

Normal biology does not make every symptom harmless.

32. Listen respectfully and help contact a trusted adult or health professional.

Do not demand personal details.

33. Treatment suitability depends on the person and professional guidance.

Do not share or prescribe medication.

34. It can violate privacy and expose someone to stigma.

Share general learning, not personal records.

35. Provide appropriate access and follow health guidance.

Hygiene support should be practical and dignified.

36. Helping access support and suitable services.

They should not require public disclosure.

37. It lacks individual clinical assessment.

It tests knowledge, not health status.

38. Discuss them with a qualified health professional.

Avoid assuming every change has the same cause.

39. Give general support routes and respectful information.

Personal stories are not required.

40. Seek prompt medical attention using appropriate local support.

Do not let the lesson delay care.

41. D. Reproductive cells

They are also called gametes.

42. C. A sperm and egg fuse

Fusion forms a fertilised egg.

43. B. Zygote

Use the introductory biological term.

44. A. An oviduct or fallopian tube

It is usually before implantation in the uterus.

45. D. No

They are distinct events.

46. C. Ovulation

Release and fusion are different.

47. B. A simplified teaching sequence

It is schematic and not diagnostic.

48. A. Detailed early division stages are outside its scope

The curriculum's scope is limited.

49. A reproductive cell such as a sperm or egg.

The two cell types have roles in fertilisation.

50. Sperm and egg.

Their fusion forms a fertilised egg.

51. Fusion of a sperm and egg.

It is not the same as implantation.

52. Fallopian tube.

This is the usual site of fertilisation.

53. Fertilised egg.

The term names the result of fertilisation.

54. The uterus.

The sequence is shown schematically.

55. Real biological processes vary and the animation is simplified.

Medical assessment is different from a lesson.

56. Release is ovulation; fusion is fertilisation.

Use separate terms for separate events.

57. It explains a process without claiming to be a literal photograph.

Labels and assumptions matter.

58. No, usually in an oviduct.

Use accurate anatomy.

59. For example, detailed hormonal control or early division stages.

The scope follows the introductory curriculum.

60. With accurate, respectful scientific language.

No personal disclosure is needed.

61. D. The uterine lining

The uterus is distinct from the usual fertilisation site.

62. C. An early embryo at the blastocyst stage

Detailed blastocyst formation is outside scope.

63. B. Fertilisation

These are stages in a possible reproductive sequence.

64. A. No

Release, fertilisation and implantation are not guaranteed.

65. D. No

It is an educational model.

66. C. Fertilisation is fusion; implantation is attachment in the uterine lining

The processes have different definitions.

67. B. Detailed early developmental stages

It follows the introductory scope.

68. A. Appropriate qualified health advice

Personal concerns need suitable services.

69. Attachment and embedding of an early embryo in the uterine lining.

It is distinct from gamete fusion.

70. Uterus.

It is not the usual site of fertilisation.

71. Blastocyst.

The detailed formation stages are not required here.

72. Fertilisation.

The sequence also includes earlier ovulation.

73. Not every fertilised egg implants successfully.

A model sequence is not a guarantee.

74. Menstruation sheds lining; implantation attaches an early embryo to it.

Use accurate process names.

75. It is a teaching symbol, not a real person's medical evidence.

A diagram cannot diagnose.

76. Fertilisation is usually in an oviduct; implantation is in the uterus.

Location helps distinguish the processes.

77. They are beyond this introductory lesson's scope.

Omission should be stated, not filled with invented stages.

78. Private health information about themselves or others.

General scientific explanations are enough.

79. It explains definitions and sequence.

Understanding a process is not assessing an individual.

80. Seek support from a trusted adult and appropriate health professional.

The app should not delay or replace care.

81. D. Yes

Many infections can be asymptomatic.

82. C. No

Symptoms may be absent or have several causes.

83. B. Gonorrhoea

It is one of the curriculum examples.

84. A. No

HIV infection and AIDS describe different things.

85. D. No

Casual contact is not a reason for exclusion.

86. C. They reduce many STI risks when used correctly and consistently, but do not eliminate every risk

Some infections can involve uncovered skin.

87. B. A qualified health service

Testing and care require appropriate support.

88. A. Support privacy and access to care

Stigma can discourage help-seeking.

89. For example, gonorrhoea and syphilis.

HIV and genital herpes are also named.

90. Many STIs can be asymptomatic.

Appropriate testing may be needed.

91. For example, unusual discharge, sores or pain when urinating.

Such symptoms do not by themselves prove an STI.

92. Infections can be invisible and symptoms are not specific.

Do not judge or diagnose people.

93. Avoiding sexual contact prevents sexual exposure; condoms reduce many risks when used correctly and consistently.

No simple classroom claim should promise elimination of every risk.

94. They can transmit infections and cause injury.

Learners should not handle sharps.

95. Promptly seek guidance from a qualified health service with trusted support.

Do not wait for an educational quiz to decide.

96. Diagnosis and treatment depend on qualified assessment.

The wrong medicine can be ineffective or harmful.

97. Use respectful language and protect privacy.

Infection is not a reason to blame or exclude.

98. Rumours do not establish status, and casual contact does not spread HIV.

Respect and safeguarding matter.

99. None about symptoms, menstrual history or sexual behaviour.

It teaches general knowledge only.

100. A trusted adult and appropriate health or safeguarding service.

Safety and qualified support take priority.

Explore responsibly

The HTML includes YouTube topic-search links, not teacher-vetted individual videos. External content may include advertising, inaccurate claims or age-inappropriate recommendations. A teacher should preview results. The original built-in animations need no internet.

Share the complete site folder, or host it with index.html at the root. Progress is stored locally and separately for each topic; there are no accounts or shared class records.

Before classroom use, review scientific accuracy, question difficulty, practical safety, accessibility and curriculum coverage. Supervised practical work is still needed where the curriculum requires it.

Official KICD curriculum listing

Original curriculum viewer (external)

NHS: periods

WHO: STI information